

Build Steps

Cowork is task-driven — give Copilot a job, optionally schedule it, let it use your M365.

1 Open Copilot Cowork

Open Microsoft 365 Copilot and switch to Cowork mode (left rail). Confirm the surfaces you have available — Cowork can act on your behalf across:

- Outlook (read, summarise, draft, send, schedule)
- Teams (chats, channels, post messages, read history)
- Calendar (find times, create meetings, prep briefs)
- SharePoint & OneDrive (read files, write Word/Excel/PowerPoint to your files)
- Power BI (semantic models, reports, DAX queries)

2 Paste the Headline Task

Open a new Cowork task and paste the prompt below. This is the worked example for Healthcare — Intake Triage Prep — adapt names, channels, and Power BI reports to your tenant before running.

Every day at 7:00am, run the Intake Triage Prep for the 7:30am clinical huddle:

1. Do NOT send/post automatically — leave the draft for me to review. All data is synthetic; treat as if it were real PHI.
2. Open Healthcare_Intake_Data.xlsx from /Clinical/Intake/ in SharePoint. Read all rows with IntakeDate = yesterday or overnight (last 14 hours).
3. For each intake, capture: PseudonymisedID, AgeBand, Sex, PresentingComplaint, DurationDays, RedFlagsDetected, RedFlagNotes, TriageOutcome, Clinician.
4. Group by TriageOutcome (Emergency → Same-day → Urgent → Routine). Within each group, list red-flag cases first.
5. For every RedFlagsDetected = Y row, write a one-sentence structured pre-consult line: '<PseudonymisedID>, <AgeBand>, <Sex>: <PresentingComplaint> x <DurationDays>d — red flags: <RedFlagNotes>. Triage: <TriageOutcome>.'
6. Detect cluster signals: if 3+ intakes overnight share the same broad PresentingComplaint category (e.g., GI, resp, chest pain), flag as 'possible cluster — clinician review'.
7. Build a Word brief saved to /Clinical/Huddle/<date>.docx: counts by TriageOutcome, the red-flag list verbatim, cluster flags, and a Clinician Workload table (intakes per Clinician).
8. Draft a Teams message to the 'Clinical Huddle' channel with the counts, cluster flags, and a link to the brief. Leave as draft.
9. Never offer diagnosis, treatment plan, or risk score. Surface, summarise, route. Any uncertainty → escalate to the on-call clinician via a direct Teams DM draft.

3 Set the Schedule

In the task panel, choose Schedule and configure:

- | | |
|-----------------------|---|
| Frequency | Daily |
| Run Time | 07:00 local (30 min before clinical huddle) |
| Execution Mode | Inline (recommended) — task remembers context across runs |

4 Verify the First Run

Run once now (Run now) before scheduling kicks in. Confirm:

- Every overnight intake is included; counts match the workbook
- Red-flag lines use the structured one-sentence format consistently
- Cluster detection fires only on genuine 3+ groupings — try planting one
- Zero diagnostic or treatment language anywhere in the brief

Try These Live

Copy any of these into a new Cowork task — one-shot or scheduled.

Red-Flag Watchlist

Across the last 7 days of intake, list every PseudonymisedID with RedFlagsDetected = Y that was triaged Routine. For each, surface the RedFlagNotes and the Clinician. Draft a Teams DM to the Clinical Lead asking for a second-look review. Never imply a diagnosis was missed — language stays neutral.

Symptom Cluster Scan

Read the last 14 days of intake. Tag PresentingComplaint into broad categories (resp / GI / cardiac / dermatology / neuro / MSK / other). Plot counts by day and call out any category trending up >50% week-over-week. Build a one-page Word brief for the public-health lead.

Clinician Load Balancer

From this week's intake, count intakes per Clinician and the share of Same-day or Emergency triage. Identify any clinician with both high volume and high acuity share. Draft a Teams DM to the rota lead with a suggested rebalance for tomorrow. Leave draft.

Patient Acknowledgement Drafts

For every Routine triage from yesterday, draft a short, plain-language acknowledgement message: complaint logged, expected next step, no clinical detail. Anonymise by PseudonymisedID. Leave drafts in my Drafts for the patient-comms team to send.

How to Think About This Teammate

Cowork replaces:

- Pre-huddle scramble to read overnight intake
- Clusters spotted only after a week of cases
- Workload imbalance discovered at week's end

With:

- ✓ A pre-staffed huddle brief by 7:00am
- ✓ Cluster signals surfaced the morning after
- ✓ Workload signals before the rota locks

Not a chatbot. A scheduled teammate with full access to your M365.

Tip for real-world use

Healthcare's hardest rule is 'never diagnose'. Pin it: 'You do not offer diagnosis, treatment, severity, or prognosis — you surface, structure, route.' If Cowork drifts into clinical reasoning, tighten the prompt — never workaround. Datasets here are synthetic.

You've built a Cowork teammate ✓

You now have a Copilot teammate that can:

- Structure overnight intake before huddle
- Spot clusters the next morning, not next week
- Route red flags to clinicians, never interpret
- Balance clinician load with real signal